

FRONT

1 cm
Barcode**hovid SETAMOL-300 Suppositories**

VISET08-M

DESCRIPTION

Milk white to faint yellow, torpedo-shaped suppository.

COMPOSITION

Each suppository contains Acetaminophen 300 mg.

ACTIONS AND PHARMACOLOGY

Acetaminophen is an aniline derivative with analgesic and antipyretic actions similar to those of aspirin but with no demonstrable anti-inflammatory activity. Acetaminophen is less irritant to the stomach than aspirin. It does not affect thrombocyte aggregation or bleeding time. Acetaminophen is generally well tolerated by patients hypersensitive to acetylsalicylic acid.

PHARMACOKINETICS

Absorption: Acetaminophen is well absorbed by both oral and rectal routes. Peak plasma concentrations occur about 2 to 3 hours after rectal administration. The plasma half life is about 2 hours.

Biotransformation: Acetaminophen is primarily metabolised in the liver by conjugation to glucuronide and sulphate. A small amount (about 3-10% of a therapeutic dose) is metabolised by oxidation and the reactive intermediate metabolite thus formed is bound preferentially to the liver glutathione and excreted as cysteine and mercapturic acid conjugates.

Elimination: Excretion occurs via the kidneys. 2-3% of a therapeutic dose is excreted unchanged; 80-90% as glucuronide and sulphate and a smaller amount as cysteine and mercapturic acid derivatives.

INDICATION

For the treatment of mild to moderate pain and pyrexia in children weighing between 15 and 24 kg (or aged between 4 to 9 years).

Acetaminophen suppositories may be especially useful in patients unable to take oral forms of acetaminophen, e.g. post-operatively or with nausea and vomiting.

CONTRAINDICATIONS

- Hypersensitivity to acetaminophen or to any of the excipients.

WARNINGS AND PRECAUTIONS

- This preparation contains acetaminophen. Do not take any other acetaminophen containing medicines at the same time. Acetaminophen should be given with care to patients with impaired kidney or liver function.
- Do not exceed the recommended dose. Doses higher than those recommended involve a risk of very severe liver damage. Liver damage is also associated with certain risk factors. If liver damage is suspected then liver function tests should be performed.

- Immediate medical advice should be sought in the event of an overdose, even if the child seems well, because of the risk of delayed, serious liver damage.
- Acetaminophen may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of serious condition. If these reactions occur, stop use and seek medical assistance right away.

PREGNANCY AND LACTATION

Not applicable.

DRUG INTERACTIONS

Drugs which induce hepatic microsomal enzymes such as alcohol, barbiturates and other anticonvulsants, may increase the hepatotoxicity of acetaminophen, particularly after overdosage.

The anti-coagulant effect of warfarin and other coumarins may be enhanced by prolonged regular use of acetaminophen with increased risk of bleeding. The effect appears to increase as the dose of acetaminophen is increased, but can occur with doses as low as 1.5-2 g acetaminophen per day for at least 5-7 days. Occasional doses have no significant effect.

Probenecid inhibits the glucuronidation of acetaminophen which can affect the clearance of acetaminophen. This should be considered when these medicines are administered concomitantly.

Acetaminophen may affect the pharmacokinetics of chloramphenicol. This interaction should be considered when these medications are administered concomitantly, especially in malnourished patients.

Enzyme-inducing medicines, such as some antiepileptic drugs (phenytoin, phenobarbital, carbamazepine, primidone) have been shown in pharmacokinetic studies to reduce the plasma AUC of acetaminophen to approx. 60%. Other substances with enzyme-inducing properties, e.g. rifampicin and St. John's wort (hypericum) are also suspected of causing lowered concentrations of acetaminophen. In addition, the risk of liver damage during treatment with maximum recommended doses of acetaminophen will be higher in patients being treated with enzyme-inducing agents.

MAIN SIDE/ ADVERSE EFFECTS

Side-effects at therapeutic doses are rare.

Common ($\geq 1/100$ to $<1/10$): *Gastrointestinal disorders:* Redness of the rectal mucous membranes

Rare ($\geq 1/10,000$ to $<1/1,000$): *Immune system disorders:* Allergic reaction; *Hepatobiliary disorders:* Liver damage; *Skin and subcutaneous tissue disorders:* Exanthema, urticaria, angioedema; *Investigations:* Increase in creatinine (mostly secondary to hepatorenal syndrome)

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Serious skin reactions including skin reddening, blisters or rash have been reported.

There have been reports of blood dyscrasias including thrombocytopenia and agranulocytosis, but these were not necessarily causally related to acetaminophen.

Hepatic necrosis may occur after overdosage.

OVERDOSE AND TREATMENT

Toxicity: Liver damage is possible in adults who have taken 10 g or more of acetaminophen. Ingestion of 5 g or more of acetaminophen may lead to liver damage if the patient has risk factors.

If the patient

- is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's wort or other drugs that induce liver enzymes, or
- is likely to be glutathione depleted e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms: Symptoms of acetaminophen overdosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after administration and clinical symptoms generally culminate after 4 to 6 days. Abnormalities of glucose metabolism and metabolic acidosis may occur.

In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Management: Immediate treatment is essential in the management of acetaminophen overdose. Despite a lack of clinically significant early symptoms, patients should be referred urgently to hospital for immediate medical attention. This is because early symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines.

As concentrations soon after acetaminophen ingestion are unreliable, plasma acetaminophen concentration should be measured at 4 hours or later after the initial administration. Treatment with N-acetylcysteine may be used for up to 24 hours after administration of acetaminophen; however, the maximum protective effect is only obtained up to 8 hours post-administration. The effectiveness of this antidote declines sharply after this 8 hour time period. If required, the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, then oral methionine may be a suitable alternative for remote areas, outside hospital.

DOSAGE AND ADMINISTRATION

- Not to be taken by mouth, as per rectal administration only.
- For children, the dosage must remain proportionate to body weight. Approximate ages provided as a guide only.
- The recommended daily dose for children is approximately 60mg per kg per day, administered in 4 to 6 doses (i.e. roughly 15 mg/kg every 6 hours).
- Due to the risk of localised poisoning, insertion of more than 4 suppositories per day is not recommended. Rectally-administered treatment should be kept as short as possible.
- Suppositories are not a suitable form of treatment in cases of diarrhoea.
- Only whole suppositories should be administered - do not break suppository before administration.
- **Children weighing between 15 and 24 kg (or aged between 4 and 9 years)**
Dosage: 1 suppository, repeated every 6 hours if necessary, for a maximum of 4 suppositories per day.
- **Maximal recommended doses:**
 - *Children with body weight less than 37 kg:* total dose of acetaminophen should not exceed the dose of 80 mg/kg/day.
 - *Children with 38 kg up to 50 kg body weight:* total dose of acetaminophen should not exceed the dose of 3 g/day.
 - *Adults and children with body weight more than 50 kg:* total dose of acetaminophen should not exceed the dose of 4 g/day.
- **Frequency of Doses**
 - Regular doses help to avoid recurrent attacks of pain or fever. For children, doses should be taken at regular intervals (including during the night) - ideally of 6 hours, and never less than 4 hours.
 - If your child has severe kidney dysfunction (with a rate of creatinine clearance below 10 ml per minute), leave at least 8 hours between doses.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage : Store below 30°C.
Protect from light and moisture.

Presentation/Packing : Blister of 1 x 6's

Product Registration Holder/Repacked by:

HOVID Bhd.

121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

Manufactured by:
Jiangsu Farever Pharmaceutical Co., Ltd.
No. 18, Yangshan Road Street Avenue, Economy Development Zone, Xuzhou City, P. R. China.

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